

[SYNTHETIC SAMPLE] Fictional record generated for the DELPHi prototype demo. Names, institutions, and statements are invented; the layout imitates a generic field-medical format. Not a real document and not derived from any real record.

[CONFERENCE REFERENCE] The meeting named above is a real, publicly announced scientific congress and is used here only as the setting of this fictional record. No session content, abstract, poster, or presented data from that meeting is reproduced or paraphrased; every person and conversation below is invented.

Congress Attendance Report — American Academy of Neurology Annual Meeting (AAN) 2025

Meeting period: April 2025 | Author: Rachel Suh

Scope: unsolicited conversations held on-site. Session content and presented data are deliberately not reproduced here.

Conversation — Silvio Quintrell, MD (Ironwood Neurology Associates)

A hallway conversation during the afternoon break. Referenced a review that circulated on the service in passing. Asked whether I would be at the meeting later this year. Noted that formulary questions takes longer than the clinical decision and that this is not specific to any one product.

Conversation — Dagmar Lindenmuth, MD (Alder Point Neurology Associates)

The conversation happened while waiting for a session room to clear. Referenced a guideline update discussed at journal club in passing. The composition of the pediatric-to-adult transfers has shifted over the last year or two.

Conversation — Saoirse Inverness, MD (Granite Bay Epilepsy Center)

A hallway conversation during the afternoon break. The referral queue keeps slipping since the summer, which shapes how much time there is for anything else. Flagged women of childbearing potential as the group with the fewest documented options.

Conversation — Eloise Cavanaugh, MD (Marbury Neurological Care Center)

We spoke standing near the exhibit aisles, briefly. Talked through long-standing patients in general terms, without any specific case. Left it at that, with a call later this month to be scheduled. Asked whether I would be at the meeting later this year.

Conversation — Josefina Abernathy, MD (Riverstone Comprehensive Epilepsy Clinic)

Stopped to talk on the way out of a session block, with no agenda on either side. Agreed to reconnect at the following quarter; no specific request. The composition of newly referred patients has shifted over the last year or two. Described how long-standing patients is triaged now compared with before.

Conversation — Imani Quintrell, MD (Northgate Neurology Partners)

Stopped to talk on the way out of a session block, with no agenda on either side. Titration scheduling is handled by nursing staff, which the practice finds workable. Referenced a session summary from a colleague in passing.

Conversation — Yesenia Kirkpatrick, MD (Cedar Ridge Regional Epilepsy Program)

Stopped to talk on the way out of a session block, with no agenda on either side. Benefit checks varies by payer, which came up as a general frustration. Described how the pediatric-to-adult transfers is triaged now compared with before. Referral patterns into this clinic have changed, and the pediatric-to-adult transfers is a bigger share than it was.

Conversation — Kiara Prendergast, MD (Saltmarsh Neurological Care Center)

The conversation happened while waiting for a session room to clear. Referral patterns into this clinic have changed, and the refractory group is a bigger share than it was. Noted that benefit checks remains the slowest step and that this is not specific to any one product.

Conversation — Valentina Kirkpatrick, MD (Fox Hollow Neurology Partners)

Caught between sessions in the poster hall; the conversation was short and entirely unprompted. Referral patterns into this clinic have changed, and long-standing patients is a bigger share than it was. Onboarding one of the nurse practitioners is the current preoccupation here.

Conversation — Rosalind Dellacroix, MD (Lakeshore Neurological Care Center)

We spoke standing near the exhibit aisles, briefly. The referral queue is slowly recovering since the summer, which shapes how much time there is for anything else. Described how long-standing patients is triaged now compared with before. Described a preference for slower titration in general, unrelated to any particular patient.

Conversation — Renata Cavanaugh, MD (Northbrook Neurology Institute)

Introduced by a colleague from the same region and talked for a few minutes. Asked about my travel schedule, since visits keep landing on the busiest clinic days. Referenced a review that circulated on the service in passing. Mentioned a review that circulated on the service but did not raise any specific finding.

Follow-ups have been logged in the interaction system.